

THE OPPORTUNITY

InPursuit Health is raising **\$3.5M in a single round of new Class C units** at a **\$20M pre-money valuation**.

Priced equity, \$25K minimum, one close. *All materials are strictly confidential.*

This round is the launch.

Engine 1 is built and in final countdown: generally available, recognized by CMS on July 27, and already listed on CMS.gov, with several of the newly announced pledge categories live in the app. The round deploys the app to the stores (~\$100K) and switches on revenue across three separate, repeatable distribution channels InPursuit already holds. The clock starts at close, not on the calendar: weeks from funding to first D2C collections, not quarters.



Weeks from launch in the App Store and Google Play: direct-to-consumer reach to the entire app market, beyond any single partner's population. The same launch also activates the three distribution channels below.

DISTRIBUTION ALREADY IN HAND



Federal Distribution

Medicare App Library**67M recipients**

JUL 27 • PRESENTED

Three CMS awards · General Availability achieved

Recognized with an award for each pledged category at the Health Tech Ecosystem one-year anniversary, signed by the Administrator of CMS. Listed on CMS.gov, with several newly announced pledge categories already in the app.



Provider Distribution

Honeycomb Medical**22,000 patients**

JUL 15 • SUBMITTED

\$300K of InPursuit technology in Mississippi RHTP applications

\$165K technology backbone plus \$135K vitals-verified telehealth, submitted by our anchor practice. Awards projected August 2026.



Employer Distribution

J. Krug Insurance Broker**~300 employer clients**

JUL 31 • DELIVERING

The app delivers for testing

The employer benefits channel gets the product in hand, the first commercial channel to hold it, weeks ahead of consumer launch.

WHAT HAS ALREADY BEEN DE-RISKED

Technology

Infrastructure built and independently reviewed. The DiMe Seal confirms the technology is real, safe, and ready.

Validation

General Availability + 3 CMS awards. Featured in the CMS showcase alongside Apple, Oura, and MEDITECH.

Distribution

Medicare App Library 67M · Honeycomb signed · J. Krug delivering.

Commercialization

This round: onboarding, marketing, revenue. The last and cheapest step: scaling.

The primary remaining challenge is commercialization execution.

THE INVESTMENT THESIS

\$3.5M, once. App live weeks from close. Both engines by Q4.

The round works on two clocks: the D2C app launches within weeks across the App Store, Google Play, and three distribution channels, while the same capital funds the months-long clinical build, NPI, credentialing, clinicians, and payer connections, that switches on cash-pay care and the Partner Network.

NOW • THE ROUND

\$3.5M

New Class C units · \$20M pre-money
\$25K minimum · priced equity · one close
D2C app launches weeks after close

NEAR TERM

Cash-pay clinical, *live in weeks*

Weight/GLP-1 and men's/women's lines patients already buy. No payer gate, no credentialing wait. Additive to the ladder.

Q4 • 2026 • INFLECTION

Revenue inflection

All-payer billing live · both engines earning
Operations increasingly carried by collections

Next round optional, not required

REVENUE PROJECTIONS

The Infrastructure That Compounds

One infrastructure, built for a person-centric healthcare ecosystem: the only one that serves both sides of the same person. For consumers, it's how you **Harness the Power of Your Data™** to generate personalized healthcare and wellness that saves time, saves money, and produces better outcomes. For providers, it's how practices deliver digital healthcare services they could never offer before, when **Results Matter™** most. The same infrastructure makes each engine stronger, and no competitor can replicate one side without becoming the whole.

Consumer Engine
~\$5.3M

App membership + cash-pay clinical care delivered directly under our own NPI

— InPursuit —
ONE INFRASTRUCTURE

Provider Engine
~\$1.248M / practice

RPM, CCM & APCM delivered through the Partner Network · all-payer, 2026 CMS rates

THE APP + THE HONEYCOMB PLAYBOOK, REPEATED

5 MEMBERS
\$9.9M
\$6.2M network + \$3.7M app

YEAR 3 BASE CASE
25 MEMBERS
\$35M
\$31.2M network + \$3.7M app

50 MEMBERS
\$66M
\$62.4M network + \$3.7M app

100 MEMBERS
\$128M
\$124.8M network + \$3.7M app

Our anchor runs ~\$2.08M. Every rung assumes members earn barely more than half of that.

members × ~\$1.248M network + ~\$3.67M app = the rung

Steady-state at full ramp · members modeled at ~10K panels, half of Honeycomb's actual ~22K · the app figure is Engine 1's app line only; its cash-pay clinical (~\$161M of the ~\$5.3M) is additive and not in the rungs · no revenue share · a model demonstrated once, found again across independent practices nationwide, with rural health funds targeting them · complete rate-by-rate build in diligence.

Upside, **excluded from the base**: iOS / Android growth, CIN participation, and D2C paid-tier upsell across every member panel, all unmodeled.

25 practices out of ~230,000

A ~\$35M run-rate needs 0.011% of the market. This is a floor, not a forecast.

THE RURAL PLAYBOOK · GRANT-FUNDED ACQUISITION

A \$50 billion federal window is open now, and we have already filed.

\$50B
RURAL HEALTH TRANSFORMATION · 5-YEAR
FEDERAL PROGRAM

\$10B / yr
AVAILABLE ANNUALLY THROUGH 2030

~\$200M
ALREADY AWARDED TO EVERY STATE, FY2026

50 states
CHOOSING TECHNOLOGY PARTNERS NOW

HOW THE PLAYBOOK RUNS, ALREADY PROVEN ONCE

- The state pays.** A rural practice gets InPursuit's infrastructure funded by Rural Health Transformation dollars, not out of its own pocket.
- The practice gains the capability.** Full care-management infrastructure stood up, and InPursuit is paid at onboarding.
- The template repeats.** Across every rural member, in any of 50 states, while the states are deciding who to spend the money with right now.

Grant-funded acquisition, non-dilutive, and the core of our rural sales motion.

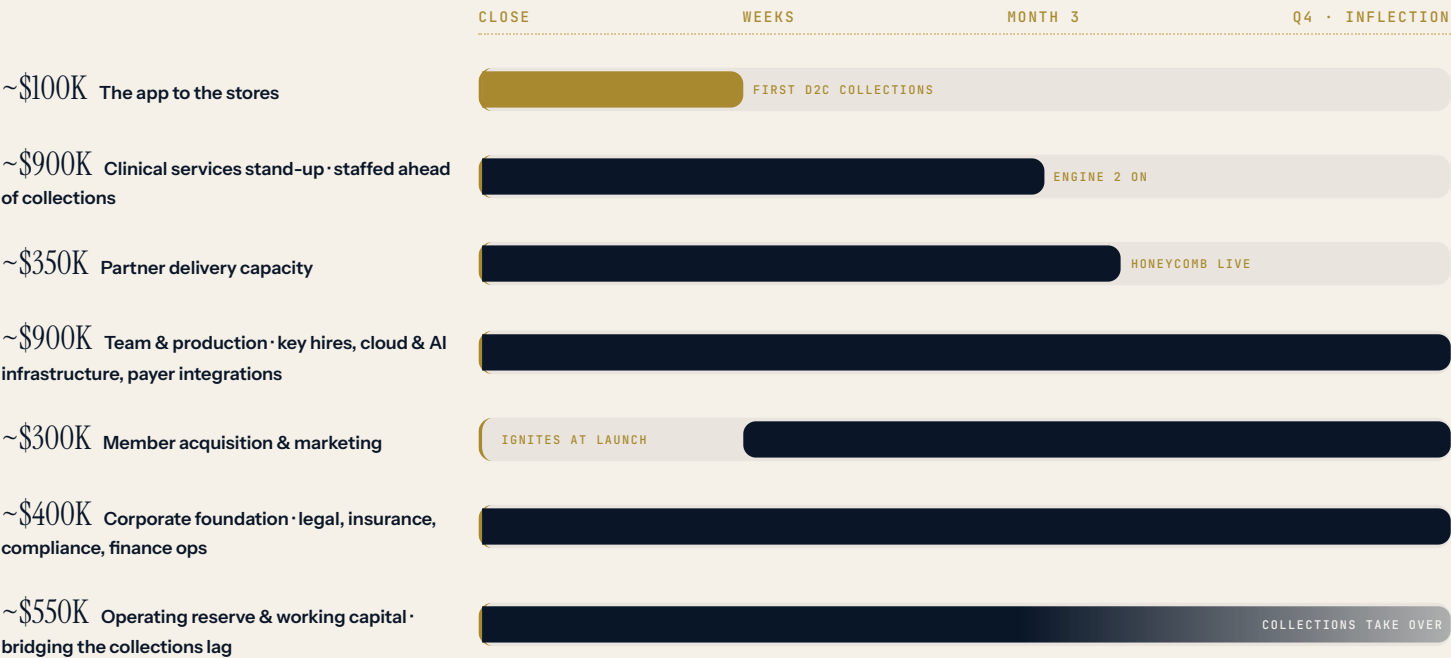
ALREADY FILED
\$300K

SUBMITTED FOR HONEYCOMB
Filed for Honeycomb's ~2,000-patient
Mississippi footprint. Awards projected
August 2026.

USE OF PROCEEDS

TOTAL ROUND \$3.5M

Everything starts on day one. Nothing waits on anything else.



WHAT YOUR POSITION LOOKS LIKE • \$20M PRE-MONEY, \$3.5M RAISE, \$23.5M POST

\$250K
~1.06%

Funds the launch that converts the round from projection to receipts.

\$500K
~2.13%

A meaningful position in the infrastructure both engines run on.

\$1M
~4.26%

An anchor stake, ahead of the Series A step-up at scale.

Ownership shown at the \$20M pre-money cap. Class C units convert at the C-Corp conversion, where QSBS eligibility begins. Illustrative; not a guarantee of returns.

TERMS

INSTRUMENT	Class C units	STRUCTURE	Priced equity	PRE-MONEY	\$20M
ROUND	\$3.5M • single close	MINIMUM	\$25K		
HOW TO PARTICIPATE					
1 Request materials at invest@inpursuithealth.com		2 Execute the subscription agreement via DocuSign		3 Wire funds • single close	

“What \$3.5M actually buys: ownership in the infrastructure for a *person-centric healthcare world*. And the capital to scale from one Honeycomb to 25 member practices on infrastructure designed for unlimited scale.”